



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : مياده مبروك ابراهيم محمود
المحافظة : الجيزة
الحى / المدينة : الهرم
رقم البطاقة : 28304232100685 رقم التليفون : 01099984793
العمر (تاريخ الميلاد) : 23-04-1983 المهنة : ربه منزل
العنوان : 000000000000
تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمدة 6 شهور
قسم العلاج :
الأورام

[14163]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
.....

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: **مياده مبروك ابراهيم محمود**
National ID: **28304232100685**



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Goserelin (Zoladex) subcutaneous

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
 وزارة الداخلية
 بطاقة هوية شخصية

مواليد
 مبروك إبراهيم مصطفى
 ٠٦ - ٠١ - ١٩٨٥
 القاهرة - الجيزة

٢٨٨٧١٠٤٢٣٢١٠٥٦٨٥
 KM6236412

٢٠٢٤/٠٣/٠٦
 ٢٨٨٧١٠٤٢٣٢١٠٥٦٨٥
 حاصلة على مقرر من مصلحة وقائي
 قسم الاشراف السياحي
 اتي
 كمال مندي ابو اسعد دويار
 البطاقة سارية حتى ٢٠٢٤/٠٣/٠٦



SCAN 8 LABS

Name : Mayada Mabrouk Ibrahim Mahmoud

Branch : Dokki

Ref.Physician Prof : Goustav Roussey Egypt

Date : 25-03-2026 09:14

Medical No. : 335712

Ins.Company : شركة مستشفيات

Tc-99m MDP SKELETAL SCINTIGRAPHY

Clinical indication: 42-year-old female; history of breast cancer; follow up.

Technique: Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings:

Comparison is made with the prior study dated January 9, 2025.

- Stable appearance of the previously reported osseous lesions at the mid-dorsal vertebra and its adjacent rib (posterior end).
- The proximal shaft of the right tibia demonstrates tracer enhancement surrounding the site of reported surgery, consistent with benign post-surgical changes.
- No other abnormal active or cold focal bony lesions are identified elsewhere.
- Degenerative and arthritic changes are noted in the spine, sternum, both shoulders, and weight-bearing joints.

Impression:

- Stable osseous metastatic disease involving the mid-dorsal vertebra and adjacent rib, with no evidence of new or progressive lesions.
- Benign post-surgical tracer uptake at the proximal right tibia.
- No scintigraphic evidence of additional osseous metastatic disease.
- Clinical correlation and follow-up as per oncology pathway.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



Patient's Name: mayada mabrouk ibrahim mahmoud
Date: 2026-03-26
Referred by: Prof.Dr. cap+e
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I. CHEST:

Known case of left breast cancer showed right breast heterogeneously enhancing mass lesion measuring 23 x 20 mm with tiny calcifications overlying skin thickening.
left axillary enlarged lymph nodes, the largest measures 18 x 8 mm.
No consolidation or collapse.
No hilar or mediastinal masses or lymph node enlargement.
No pleural or pericardial collection.
Normal plain CT appearance of the heart and great vessels.
D9 and adjacent left posterior rib large mixed predominantly sclerotic lesion with subsequent hyperostosis

II. ABDOMEN AND PELVIS:

Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
The urinary bladder shows no evidence of gross masses or diverticulae or stones.
Normal appearance of the pelvic organs with no pelvic masses or collections.
No abdominal or pelvic lymphadenopathy.
No ascites.

Opinion:

- ❖ Known case of left breast cancer showed left breast mass lesion with left axillary lymph nodes.
- ❖ D9 and left posterior rib mixed sclerotic lesion likely metastatic

Dr. Ayser Ahmed



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
مصرية
طبية

MDT FORM

10 / 1 / 2023 G

Name	ياسمين محمد علي	MRN	3705-9.99
Age	40 years	ID	2830.25351-70
Menopausal status		Phone	01099982792
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	DM / HTN	REASON OF MDT	

BX:

Clinical Data

ER:

+ve

PR:

+ve

HER 2:

-ve

Ki67:

R

L

Sonomammography:

Mets Workup:

CT CAP → prec



TNM

Recommendations:

CT chrt → NAD

Mets breast Ca to bone Since 4/22 → femur
Mixed response → shift
to EDK4/6 + Zoladex - pt shifted to 10/1/23

MDT OUTCOME & PLAN

for Postmenopausal + Lence / Tamoxifen

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

SMC-DSH-F-MDT-001

Revision no:0

Issue no:1

Issue date: 1/9/2022

HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION AND DECISION SUPPORT



الهيئة العامة للتأمين الصحي
الإدارة المركزية لتكنولوجيا المعلومات
ودعم القرار

جهة الإصدار: فرع الجيزة

الى من يهمه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للارتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات والارامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة. وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعية وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعي والبحث تبين ان

السيد (ة) مياده مبروك ابراهيم محمود بطاقة رقم 28304232100685
وطبقا الى خطاب من التأمينات الاجتماعية يفيد عدم الانتفاع
يعتبر من الفئات غير الملتفحة بالتأمين الصحي
وشكرا



تحريرا في: 20-4-2022

مركز نظم المعلومات
يعتمد،،،

مستند 97678



Patient Name	: الأستاذ / ميادة ميروك ابراهيم محمود	Request No.	: 13006715
Patient No.	: 28304232100685	Request Date	: 28/09/2022
Path No.	: SR-747/22	Verification Date	: 05/10/2022
Sex	: Female	Financial Category	: اوام دار السلام - نفقة الدولة
Age	: 39 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Ali mostafa

Pathology Report

CLINICAL DATA

Known case of breast mass, presented with Multiple bony lytic lesions

NATURE OF SPECIMEN

Slide Revision (Curretage & IHC slides)

GROSS EXAMINATION

5 blocks labeled as (632167) and 16 slides.

MICROSCOPIC EXAMINATION

Examination of the slides received revealed bony tissue showing clusters of tumor cells floating in pools of extracellular matrix separated by fibrous septa of likely mammary origin

IPT

- *GATA3:positive
- *ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
- *PR: Strong nuclear reaction in 65% of tumor cells (Allred score 7/8)
- *HER2: No membrane reaction in tumor cells (score 1)
- *Ki 67: 25% nuclear reaction of tumor cells
- *CK7:positive
- *CK20:negative
- *PAX8:Negative
- *CDX2:Negative

DIAGNOSIS

Known case of breast mass, presented with Multiple bony lytic lesions , Slide revision;
Metastatic mucoid carcinoma of likely breast origin.

- * ER: Positive (score 8/8).
- * PR: Positive (score 7/8).

Verified by :

Dr.Marwa mahmoud

Printing Date : 05/10/2022 12:25:17

Page 1 of 2

Printing By : .marwa mohamed youss

Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأمثلة / ميادة مبروك ابراهيم محمود	Request No.	: 13006715
Patient No.	: 28304232100685	Request Date	: 28/09/2022
Path No.	: SR-747/22	Verification Date	: 05/10/2022
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 39 Y	Department	: Out Patient
Referring Doc :		Registered by	: Ali mostafa

Pathology Report

- * HER2: Negative (score 1+).
- * Ki67: Positive in 25% of tumor cells

Prof. Amal Hareedy

Verified by :
Dr.Marwa mahmoud

Printing Date : 05/10/2022 12:25:17

Page 2 of 2

Printing By : marwa mohamed youss

مركز الباثولوجي - مستشفى دار السلام - القاهرة
Room No. 405 - First Floor - Building 24 - 225 - El-Dokki - Giza - Egypt
K42 04

Patient Name: Mayada Mabrouk Ibrahim Mahmoud	Date : 07/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 42 Year(s)
Medical ID: 3652/2022	Pat. ID: 20480

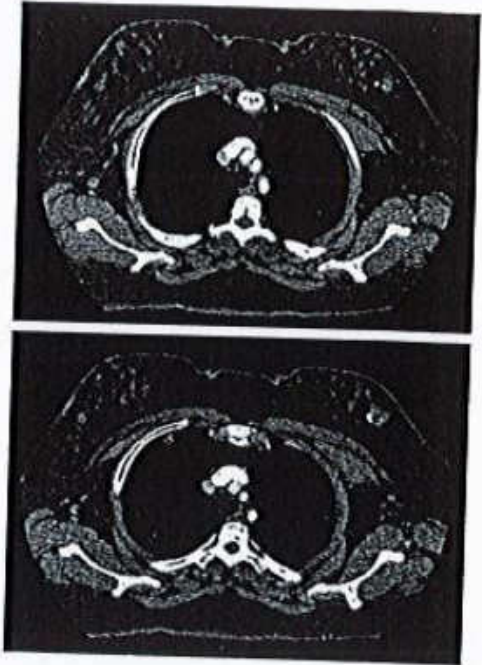
POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Clinical History:

- Known case of breast cancer, compared to the previous study report dated 24-3-2025, the current study revealed:

Findings:

- Still noted left breast soft tissue mass lesion showing calcifications currently measuring 16x23 mm.
- Still noted prominent axillary and subpectoral lymph nodes, the largest measuring 20x13mm.
- Still noted few subcentimetric left internal mammary lymph nodes.
- The right breast shows nodular pattern with retracted nipple and skin thickening reaching 10 mm for correlation with the previous unavailable films.
- Few right axillary and subpectoral lymph nodes are noted, the largest seen measuring 20x13 mm.
- No sizable nodules, masses or consolidative patches.
- No sizable hilar or mediastinal lymph nodes.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.



OPINION:

- Still noted left breast soft tissue mass lesion with prominent axillary and subpectoral lymph nodes.
- Still noted few subcentimetric left internal mammary lymph nodes.
- Right breast nodular pattern with retracted nipple and skin thickening as well as few right axillary and subpectoral lymph nodes for correlation with the previous unavailable films and sonomammography correlation.

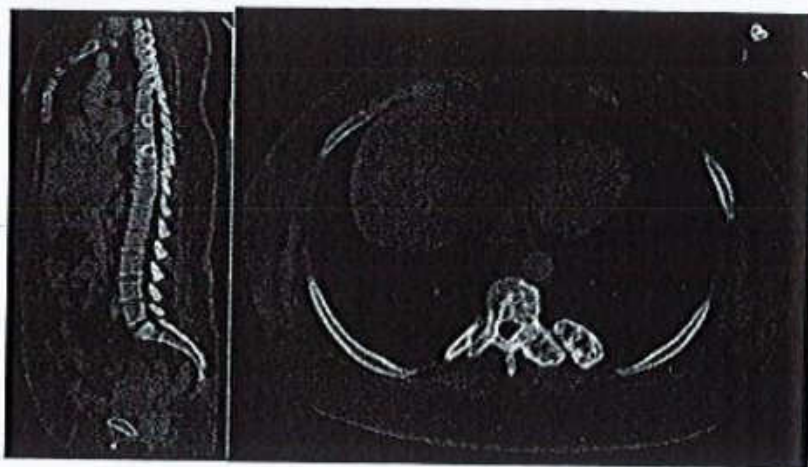
MULTI-SLICE POST CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: -

Clinical History:

- Known case of breast cancer, compared to the previous study report dated 24-3-2025, the current study revealed:

Findings:

- The liver is still enlarged (21.5 cm) with low homogenous CT texture with stationary segment IVb hyperdense area (likely area of fat sparing). No definite sizable focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Prominent abdominal and pelvic lymph nodes are noted at the portocaval, gastrohepatic, mesenteric, bilateral common iliac and external iliac, the largest seen at the right common iliac group measuring about 17x13 mm.
- Normal size (12.5 cm) and parenchymal texture of the spleen with no focal lesions seen. Lower polar splenule is noted (11x8.5 mm).
- Normal size and parenchymal thickness of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Inadequate filling of the urinary bladder with no definite masses or stones.
- No ascites.
- Fat containing umbilical hernia is noted.
- Rather stationary appearance of D5, 7, 8 and 9 osseous lesions with cortical destruction the largest still seen at D9 extending to left transverse process and posterior aspect of left 9th rib encroaching upon the spinal canal.



Opinion:

- *Prominent abdominal and pelvic lymph nodes for clinical correlation.*
- *Otherwise, stationary course of the disease process.*

Marina Essam, MSc
Dr.Noura Nabil,MD

Signature Noura Nabil
10/08/2025 1:59 AM

Patient Name: MAYADA MABROUK IBRAHIM DOB: 02-Feb-1981 ID: BONE SCAN SEX: F
STUDY: Bone Scan STUDY DATE: 02-Feb-2025

Study Name: Bone Scan
Series Date: 02-Feb-2025

Study Date: 02-Feb-2025

Series Description: Wholebody

Patient ID: BONE SCAN

Study Name: Bone Scan

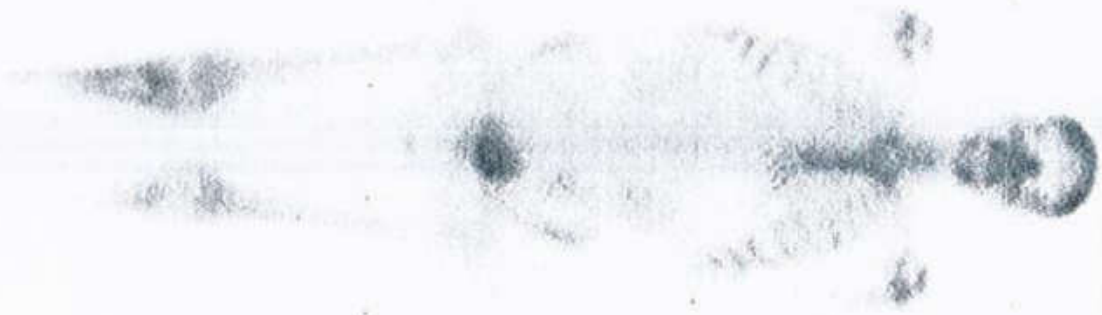
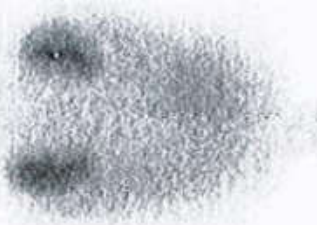
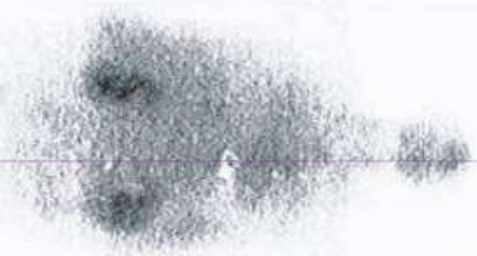
Series Description: Wholebody bone

DOB: 02-Feb-1981

Study Date: 02-Feb-2025
Series Date: 02-Feb-2025

Wholebody bone 02-Feb-2025

Wholebody 02-Feb-2025



Patient's Name:

Mayada Mabrouk Ibrahim Mahmoud

Date:

2026-08-29

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Examination:

■ Computed tomography of the chest, abdomen and pelvis

Technical data:

- 5 mm CT scans of the abdomen and pelvis with IV contrast

Clinical data: known left breast cancer

Comparison: 26-03-2026

Findings:

Chest findings:

- Left breast parenchymal mass lesion 23x20 mm with foci of calcifications
- Stationary D5, D7 , D8 and D9 and left 9th rib posterior aspect bony lesions
- Clear lungs fields with no active parenchymatous infiltrates.
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Average cardiac size, no pericardial effusion.

Abdomen and pelvis findings:

- The liver shows enlarged size with smooth contours, diffuse low hepatic parenchyma density (enlarged fatty liver) and no evidence of focal lesions.
- No intra hepatic biliary dilatation.
- Normal caliber and homogenous enhancement of the portal vein.
- The spleen shows normal size. No evidence of focal lesions.
- Normal CT appearance of the gall bladder, pancreas and adrenal glands
- Both kidneys are of normal site, size and shape, no stones, back pressure, cysts or masses bilaterally.
- The urinary bladder shows average distention, no wall thickening, no stones, diverticulae or masses.
- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction
- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection

Opinion:

- **Left breast parenchymal mass lesion 23x20 mm with foci of**

Patient's Name:

Mayada Mabrouk Ibrahim Mahmoud

Date:

2026-08-29

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

calcifications

- * Stationary D5, D7, D8 and D9 and left 9th rib posterior aspect bony lesions
- * Enlarged fatty liver

Recommendation:

- * Post manangement follow up

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Ahmed

Patient's Name: Mayada Mabrouk Ibrahim Mahmoud
Date: 2026-08-29
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Examination:
■ Computed tomography of the chest, abdomen and pelvis

Technical data:
▪ 5 mm CT scans of the abdomen and pelvis with IV contrast

Clinical data: known left breast cancer

Comparison: 26-03-2026

Findings:

Chest findings:

- Left breast parenchymal mass lesion 23x20 mm with foci of calcifications
- Stationary D5, D7 , D8 and D9 and left 9th rib posterior aspect bony lesions
- Clear lungs fields with no active parenchymatous infiltrates.
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Average cardiac size, no pericardial effusion.

Abdomen and pelvis findings:

- The liver shows enlarged size with smooth contours, diffuse low hepatic parenchyma density (enlarged fatty liver) and no evidence of focal lesions.
- No intra hepatic biliary dilatation.
- Normal caliber and homogenous enhancement of the portal vein.
- The spleen shows normal size. No evidence of focal lesions.
- Normal CT appearance of the gall bladder, pancreas and adrenal glands
- Both kidneys are of normal site, size and shape, no stones, back pressure, cysts or masses bilaterally.
- The urinary bladder shows average distention, no wall thickening, no stones, diverticulae or masses.
- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction
- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection

Opinion:

- Left breast parenchymal mass lesion 23x20 mm with foci of

Patient's Name:

Mayada Mabrouk Ibrahim Mahmoud

Date:

2026-08-29

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

calcifications

- * Stationary D5, D7, D8 and D9 and left 9th rib posterior aspect bony lesions
- * Enlarged fatty liver

Recommendation:

- * Post management follow up

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Ahmed



SCAN 8 LABS

مركز النيل

Name : Mayada Mabrouk Ibrahim Mahmoud

Date : 25-03-2026 09:14

Branch : Dokki

Medical No. : 335712

Ref.Physician Prof : Goustarv Roussey Egypt

Ins. Company : شركة مستشفى

Tc-99m MDP SKELETAL SCINTIGRAPHY

Clinical indication: 42-year-old female; history of breast cancer; follow up.

Technique: Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings:

Comparison is made with the prior study dated January 9, 2025.

- Stable appearance of the previously reported osseous lesions at the mid-dorsal vertebra and its adjacent rib (posterior end).
- The proximal shaft of the right tibia demonstrates tracer enhancement surrounding the site of reported surgery, consistent with benign post-surgical changes.
- No other abnormal active or cold focal bony lesions are identified elsewhere.
- Degenerative and arthritic changes are noted in the spine, sternum, both shoulders, and weight-bearing joints.

Impression:

- Stable osseous metastatic disease involving the mid-dorsal vertebra and adjacent rib, with no evidence of new or progressive lesions.
- Benign post-surgical tracer uptake at the proximal right tibia.
- No scintigraphic evidence of additional osseous metastatic disease.
- Clinical correlation and follow-up as per oncology pathway.

Much oblige

Dr. Alaa Nagi, MD

Nuclear medicine



الملكة رانيا الحسينية
مملكة البحرين
الملكة رانيا الحسينية

الملكة رانيا الحسينية
مملكة البحرين
الملكة رانيا الحسينية

MDT FORM

10/11/2023 G

Name	Fatma Al-Masoudi			MRN	705-550		
Age	40 years			ID	100-55501-70		
Menopausal status				Phone	100-55501-70		
MRS SCORE				Marital status	100-55501-70		
BMI				Family history			
CO morbidity	Dx: HTN			REASON OF MDT			
BX:	Clinical Data						
ER:	PR:	HER2:	Ki67:	R	L		
+	+	+	+				
Sonomammography:							
Mets Workup:							
ECT CAP → Prec							
Recommendations:				TNM			
CI checks NAD							
Met breast ca to bone Since 4/22 → Jena							
Mixed response → shift							
to EDRK 16 + Zoladex							
MDT OUTCOME & PLAN							
Pr Postheral/relaxant TL care/rommation							
SIGNATURE							
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric		
SD		Reza Al					
10/11/2023		10/11/2023					

Patient Name	: محمد علي محمد علي / م.م.م.
Patient No.	: 28304232100685
Path No.	: SR-747122
Sex	: Female
Age	: 39 Y
Referring Doc.	:
Request No.	: 13006715
Request Date	: 28/09/2022
Verification Date	: 05/10/2022
Financial Category	: All mostafa
Department	: Out Patient
Registered by	: All mostafa

Pathology Report

CLINICAL DATA

Known case of breast mass, presented with multiple benign cystic lesions.

NATURE OF SPECIMEN

Slide Revision (Currentage & HC slides)

GROSS EXAMINATION

5 blocks labeled as (632167) and 18 slides.

MICROSCOPIC EXAMINATION

Examination of the slides received revealed bony tissue showing clusters of tumor cells floating in pools of extracellular matrix and stained by fibrous septa of likely hematoma origin.

^bGATTA3:positive

ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)

¹PR: Strong nuclear reaction in 65% of tumor cells (Allred score 7/8)

HER2: No membrane reaction in tumor cells (score 1)

- KI 67: 25% nuclear reaction of tumor cells

CK7:positive

OK20: negative

⁴ PAX8: Negative

-CDX2: Negative

DIAGNOSIS

Known case of breast mass, presented with multiple bony lytic lesions, slide revision. Metastatic mucoid carcinoma of likely breast origin.

ER: Positive (score 8/8).

PR: Positive (score 7/8).

Verified by:

Dr. Manya Mahmood